Health Insurance Cashless Claims

Why are authorization requests taking longer than expected?

PROJECT Cashless Authorization / Health Insurance | **DISCOVERY STAGE** Initial - customer validation pending | **OWNER** FDE / Customer Discovery Team

01 CUSTOMER & STAKEHOLDERS

Who owns the problem, who uses the process, and who decides?

CASE CONTEXT Hospital submissions, insurer operations, policyholders and hospital teams are involved.

TOP 3 DISCOVERY QUESTIONS

- **U1** Who owns end-to-end authorization turnaround and makes the final decision?
- **U2** Which teams review, request clarification, approve, or escalate a case?
- **U3** Who represents hospitals, policyholders, operations, clinical review and IT in discovery?

02 BUSINESS OBJECTIVE

Why change? What triggered the request? What matters most?

CASE CONTEXT The COO wants verified causes and business value before any new investment.

TOP 3 DISCOVERY QUESTIONS

- **U1** What changed in the past few months: complaints, volumes, case mix or expectations?
- **U2** Which priority comes first: patient uncertainty, hospital trust, speed or operating effort?
- **U3** What business outcome and evidence would justify investing in improvements?

03 EXPECTED OUTCOMES

What should improve, and how will success be measured?

CASE CONTEXT Desired improvements are clear, but SLA definitions and targets are not provided.

TOP 3 DISCOVERY QUESTIONS

- **U1** What turnaround SLA applies to each request type, and when does the clock start or stop?
- **U2** Which targets matter for median and 90th-percentile decision time and SLA compliance?
- **U3** What reduction in pending cases, repeat status calls and complaints would count as success?

04 CURRENT PROCESS / AS-IS

What happens from request to decision? Where are the handoffs?

CASE CONTEXT Hospitals submit requests; the insurer reviews and communicates a decision.

TOP 3 DISCOVERY QUESTIONS

- **U1** Can we trace a recent delayed request from hospital submission to final decision?
- **U2** What are the exact checks and handoffs across hospital, operations and reviewers?
- **U3** How are missing documents, medical questions, resubmissions and escalations handled?

05 CORE PROBLEM & PAIN POINTS

What is going wrong? Which causes are proven?

CASE CONTEXT Delays, repeated hospital enquiries and manual work are reported; causes are unknown.

TOP 3 DISCOVERY QUESTIONS

- **U1** At which exact stage do delayed requests wait, based on case histories and timestamps?
- **U2** Which exception types recur most often, and what evidence shows their frequency?
- **U3** For the latest delayed case, what alternative causes must be ruled out before blaming systems or staffing?

06 BUSINESS IMPACT & BASELINE

How often does it happen, and what time, cost or risk results?

CASE CONTEXT Complaints are increasing; volumes, turnaround distributions and costs are unmeasured.

TOP 3 DISCOVERY QUESTIONS

- **U1** How many requests arrive by day, hospital, urgency and request category?
- **U2** What are current median/P90 turnaround, breach rate, backlog size and backlog age?
- **U3** How much handling, rework, follow-up and escalation effort does each delay create?

07 DATA, SYSTEMS & CONSTRAINTS

What systems, data, rules and dependencies exist?

CASE CONTEXT Existing digital platforms are used, but their workflow and data capabilities are unclear.

TOP 3 DISCOVERY QUESTIONS

- **U1** Which policy, claim, hospital and communication systems handle each step?
- **U2** What case IDs, status histories, timestamps and anonymized samples can be inspected?
- **U3** Which medical review, privacy, consent, security, audit and escalation rules must be preserved?

08 REQUIREMENTS & MVP OPTIONS

What capabilities could help? What is the smallest useful pilot?

CASE CONTEXT No technical solution is approved; the intervention must follow validated evidence.

TOP 3 DISCOVERY QUESTIONS

- **U1** Which confirmed bottleneck could be improved first through process or existing-system changes?
- **U2** Would validation rules, work queues, status visibility or assisted review solve that bottleneck?
- **U3** Which bounded pilot has clear users, controls, baseline, target and rollback criteria?

09 VALIDATION & NEXT STEPS

What must be confirmed, by whom, and by when?

CASE CONTEXT Root causes, commercial impact, detailed workflow and ownership require validation.

TOP 3 DISCOVERY QUESTIONS

- **U1** Who will provide anonymized case samples, event logs and a recent delayed-case walkthrough?
- **U2** Who will sign off workflow, SLA definitions, data safeguards and pilot success measures?
- **U3** When will the team review evidence, compare options and approve or reject a pilot?

EVIDENCE RULE CASE CONTEXT = assignment-stated or customer-reported (not independently verified) | U = unknown / question for validation | Do not assume the root cause or preselect AI.

SOURCE Testleaf FDE Health Insurance Customer Discovery Home Assignment and Week 5 Day 9 Customer Discovery training. Reference example: linked GitHub invoice-processing canvas.